CHAPTER

4 Essential Crisis Intervention Skills

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PREVIEW

This chapter provides an overview of the fundamental skills for effective crisis intervention work. The skills discussed here focus on Ivey et al.'s (2022) microskills hierarchy. At the heart of this hierarchy is the basic listening sequence, an interrelated set of skills that not only foster the development of rapport with clients but also aid in the identification of interventions to help achieve a successful resolution to the client's crisis state. Examples of the skills in use as well as practice exercises to foster individual skill development are provided.

LEARNING OBJECTIVES

Upon completion of this chapter, the reader will be able to:

4.1 Apply essential crisis intervention microskills.
4.2 Identify basic attending skills including eye contact, body position, vocal tone, and use of silence.
4.3 Identify the basic listening sequence including asking open and closed questions and reflecting skills.
4.4 Describe basic deescalation skills to help clients regulate emotions.

ESSENTIAL CRISIS INTERVENTION MICROSKILLS

Learning Objective 4.1 Apply essential crisis intervention microskills.

A reasonable question at this point is, "What is the difference between basic counseling skills and crisis counseling skills?" Simply put, the difference is less about what the counselor does or the techniques and strategies employed and more about contextual application, or where and in what situation skills are applied. The essential foundational skills necessary for effective crisis intervention work are indeed the basic tools on

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which the success of interventions may depend, whether the counselor is doing crisis or traditional counseling. It is important to stress that these skills will help create the counselor–client relational conditions necessary for positive change (i.e., therapeutic alliance). The skills covered in this chapter are *basic* counseling skills applied to crisis situations, and these basic skills form the foundation for the use of more advanced counseling skills. For example, just as is the case in traditional counseling, when working with clients in crisis, it is essential to listen, be empathic, and respond skillfully. Counselors also must make sure the information received from the client is accurate to create an appropriate treatment plan or referral for the client.

That said, a primary difference in crisis and traditional counseling is that crisis counseling is time-limited or at least limited to the time required to stabilize the crisis situation. That may take a single session required to hospitalize an actively suicidal client or several brief sessions in the field when helping someone cope with the ramifications of a natural disaster. Thus, crisis counseling tends to be more directive than traditional counseling (depending on the theoretical approach used) and can involve information giving, probing questions, and even direct or indirect confrontations. Finally, the focus is on the immediate issue that is causing the instability for the client so the counselor must differentiate immediate issues (risk) that are the priority from longer-term issues that may be documented for postcrisis therapy.

These basic skills provide the client with alliance-building constructs such as empathic understanding, genuineness, and acceptance and greatly facilitate the development of a safe therapeutic environment. These basic skills also aid in establishing rapport and developing an effective therapeutic alliance with the client. Rapport can be understood as a harmonious or empathetic relationship. In crisis intervention work, the development of rapport starts with the initial contact and continues throughout the process. The crisis counselor's primary concern should be fostering this rapport to develop a cohesive and supportive relationship with the client. How crisis counselors conduct themselves is crucial, as this may be the client's first encounter with a counselor and this interaction may either encourage or discourage the client from seeking counseling in the future or from following up when a proposed crisis plan is developed. The crisis counselor's ability to gain a client's trust is paramount in a crisis intervention since the counselor must ask the client sensitive questions to gather information for referrals or crisis planning.

As noted earlier, the skills covered in this chapter focus on Ivey et al.'s (2022) microskills hierarchy—a set of verbal and behavioral responses that facilitate the process of counseling and alliance formation, regardless of the crisis counselor's theoretical orientation. For some, this chapter may be a review of the basic skills taught in a skills course, while for others it may be the first consideration of these skills. Either way, it is always important to continuously be aware of and apply effective skills, since doing so is the hallmark of effective crisis counseling.

Ivey et al. (2022) present these skills as a hierarchy that is organized in a systematic framework. At the bottom of the hierarchy are the basic attending skills, such as patterns of eye contact, body position, vocal tone, and silence. A bit farther up the skills hierarchy is the basic listening sequence, which includes asking open and closed questions, using reflecting skills, and summarizing. In this chapter, each of these basic skills is reviewed, along with practical examples of the skills in use.

ATTENDING SKILLS

Learning Objective 4.2 Identify basic attending skills including eye contact, body position, vocal tone, and use of silence.

Good communication involves more than just verbal content; crisis counselors communicate with more than just words. Most meaningful communication takes place nonverbally. The next time you engage someone in conversation, take a moment to pay attention to all the nonverbal cues your partner is giving you. What does the facial expression say to you? What is conveyed by the eyes? Does your partner have a closed or open body stance? Although important in social relationships, these attending skills are even more important in the counseling relationship. Bedi (2006) surveyed clients who received counseling and asked them to identify the specific counselor behaviors that most helped to form a working alliance. Following validation and education, clients ranked nonverbal gestures, presentation, and body language as the next most important alliance-building factors. These nonverbal attending behaviors communicate a counselor's interest, warmth, and understanding to the client; they include eye contact, body position, tone of voice (vocal tone), and silence.

Eye Contact

Maintaining good eye contact is how a crisis counselor conveys interest, confidence, and involvement in the client's story (Egan & Reese, 2019). Through eye contact, clients know a counselor is focusing on them and fully committed to the helping process. Moreover, for those clients who have difficulty with closeness, making eye contact can be an important vehicle of change. However, good eye contact is not the same as staring your client down. There should be natural breaks in eye contact; eye contact should be more of an ebb and flow as you collect your thoughts and listen to the client's story.

In addition, it is essential to be sensitive to differences in how eye contact is traditionally expressed across cultures. For example, whereas direct eye contact is usually interpreted as a sign of interest in the White culture, some Asian and Native American groups believe direct eye contact is a sign of disrespect (Hays & Erford, 2023). Some Black clients may maintain greater eye contact when talking and less eye contact when listening, and many Black men will not look directly into the eyes of an authority figure because in the Black cultural context avoiding eye contact shows respect for authority figures (e.g., parents, teachers, older people).

For clients who are overly fragile or under much stress and pressure, direct eye contact may increase their level of anxiety. For example, imagine counseling a client in crisis, a 20-year-old college student who states she was sexually assaulted last night. As she tells you what happened, you notice she stares down at the floor and often avoids your eye contact. Should you confront her on her lack of eye contact and ask her to look at you directly when she speaks? Of course not. Her lack of eye contact is probably due to her feelings of shame and anxiety and is serving an emotion-regulation function. In a crisis, especially if a client is feeling overwhelmed or ashamed or is experiencing paranoia, be aware that a client who avoids your eyes is probably doing so as a protective mechanism. This reaction is normal in a crisis.

So how do you determine how much eye contact to maintain with a client? Unfortunately, no universal rule or criterion exists for what is considered either appropriate or inappropriate eye contact. As already noted, eye contact varies among cultures and is influenced by the intensity of emotions involved in the crisis situation. A good guideline to follow is to maintain a moderate amount of eye contact while monitoring your client's level of comfort and to adjust your eye contact accordingly (Young, 2017). Also, it is helpful for you to become aware of your own attending behavior so you can understand how this behavior may affect the counseling relationship. Professional counselors must be particularly aware of their use of eye contact during crisis counseling sessions. It is important to use eye contact and other nonverbal attending behaviors to reduce anxiety and deescalate anger and agitation. Complete Activities 4.1 and 4.2 to gain a deeper understanding of your own attending behaviors.

ACTIVITY 4.1

Form pairs. One person takes the role of listener, while the other takes the role of speaker and is to talk about anything of interest for about 5 minutes. During this time, the listener should maintain eye contact with the speaker as would occur normally in everyday conversation. After 5 minutes, each pair should process the experience. What feedback does the speaker have regarding the listener's level of eye contact? Was it too much? Darting? Too little? Empathic? What was most comfortable? Based on the feedback, do the exercise again, but this time have the listener try to incorporate some of the feedback received about level of eye contact. Process the activity again, and then switch roles.

Body Position

As with eye contact, your body position should convey to the client your interest and involvement. Face the client and adopt an open, relaxed, and attentive body posture, as this will assist in putting your client at ease. Counselors should not cross arms and legs and should not sit behind a desk or other barrier. In addition, Egan and Reese (2019) recommend that the counselor lean slightly toward the client, as this communicates that the counselor is listening to the client and interested in what the client has to say. Slouching in a chair or leaning away from the client may be perceived by the client as lack of interest or boredom.

The physical distance between counselor and client should also be taken into consideration; getting too close can be overwhelming and uncomfortable, whereas being too distant can make the counselor appear aloof and may be awkward for the client. Although in Western cultures the average physical distance for conversation is typically 2 to 4 feet, this comfort zone will vary from client to client (Young, 2017). When in doubt, you could let the client arrange the chairs at a distance that feels comfortable. However, keep in mind that this option is not always desirable. For example, in a crisis setting, you may be faced with a client who is paranoid or experiencing homicidal or suicidal ideation. Would you want to be within hitting or kicking distance of this client? Also, it would not be a good idea to have this client sitting between you and the door. Use your best judgment. By considering body position, you can create a safe environment for both you and your client. Your safety should always be a priority. You should never do anything that makes you

feel uncomfortable or jeopardizes your physical safety. Use your body posture and physical space to comfort clients and deescalate the anxiety, anger, or agitation a client in crisis might feel.

ACTIVITY 4.2

This activity will help you become aware of how your clients might perceive your overall pattern of nonverbal communication. Begin by forming pairs. One person will be the communicator and will face the other person, who will serve as the mirror. For the next 5 minutes, the communicator can talk about anything and, throughout this time, the mirror is to nonverbally mirror each gesture, facial expression, eye contact, and movement of the communicator. It is important that the mirror not attempt to interpret the message that is being sent by the communicator but just to mirror the perceived nonverbal communication. At the end of 5 minutes, each pair should process the experience. What was it like to see your nonverbal communication mirrored back to you? Did you learn anything about how you come across to others? Is there anything you would want to change or do more of? Switch roles and process the activity again.

Vocal Tone

Have you ever had an experience when you are engaged in conversation with someone and find yourself becoming increasingly anxious and tense, regardless of the topic? The next time this happens, pay attention to your partner's tone of voice, for you may be unconsciously responding to the emotional tone conveyed in your partner's voice. Emotions are frequently conveyed via tone of voice. The pitch, pacing, and volume can all influence how a client responds emotionally to a crisis counselor.

There is much to be said for a calm and soothing voice in times of distress, especially when the client is in a crisis. Do not underestimate the power of this attribute; your control and calmness may be among the greatest benefits to your client in crisis. Your voice can do much to help create a soothing and emotion-regulating atmosphere for the client. Learn to use your voice as a therapeutic tool. For example, if your client is overly agitated, it is often helpful to speak more slowly and in a soothing tone, as this will help your client to slow things down and begin to focus.

Also be aware of your own internal process and how your tone of voice is conveying your *own* emotional state when working with a client in crisis. It is not uncommon for your own rate of speech to increase when you hear something that causes you to feel anxious, such as a client who discloses homicidal ideation to you. If your client is agitated and using pressured speech and your speech also begins to race, your rate of speech could exacerbate your client's state.

To convey a sense of empathic understanding, it can be helpful to emphasize specific words used by your client. This technique of giving increased vocal emphasis to certain words or short phrases is called *verbal underlining* (Ivey et al., 2022). For example, consider the difference between "You were very hurt by your husband's actions" and "You were *very* hurt by your husband's actions." In the latter comment, the counselor places the emphasis on the word *very* to help reflect the intensity of the client's experience. Again, modulate your voice tone to comfort clients and deescalate the anxiety, anger, or agitation clients in crisis might feel. Now complete Activity 4.3, which considers the vocal subtleties in the spoken word.

ACTIVITY 4.3

In small groups, assign one person to be the speaker. Instruct the speaker to talk in a normal tone of voice for a few minutes about anything of interest. Have the other group members close their eyes as they listen to the speaker, paying close attention to the tone of voice, pacing, volume, and so on. After 2 or 3 minutes, stop and have the listeners give feedback on the speaker's voice. What was their reaction to the tone, volume, accent, rate of speech, and other characteristics? After this processing, repeat the activity, but this time have the speaker make changes in voice tone, volume, or pacing to deliberately create different reactions. How did the listeners respond to the changes in vocal qualities? What were the listeners' emotional responses to the various vocal qualities? Finally, have the listeners imagine themselves as a client in crisis. What types of vocal qualities would they prefer to hear?

Silence

Beginning crisis counselors often have difficulty using silence with clients. They want to keep talking to fill in any lapses or void in the session, usually to mollify their own anxiety and discomfort with a client in crisis. Or worse, counselors will engage in a monologue or resort to lecturing or teaching the client. However, constantly intervening or throwing a barrage of questions at a client in crisis can feel overwhelming and intrusive to a client who is already feeling overwhelmed. Clients in crisis need space to think, sort out their thoughts, and process what is happening. The counselor needs that time as well. This point cannot be overemphasized: Silence is important for both clients and professional counselors. It is essential for crisis counselors to develop comfort with silent pauses.

Many beginning counselors are afraid to use silence because they believe silence conveys their incompetence as a counselor. However, the opposite is true. Silence, if used appropriately and at the right time, can convey acceptance and empathic understanding. In essence, the message of silence is "I am here for you if you need me, yet I respect your need to take your time with this." Review Case Study 4.1, which looks at the use of silence in a crisis interview.

CASE STUDY 4.1

Using Silence in Counseling

The following exchange between Aden and the counselor illustrates how important silence can be in a crisis. Aden is an adult who presented to counseling with severe symptoms of depression, suicidal ideation, and an attempted suicide yesterday with an overdose of sleeping pills.

ADEN: I just want to die. All of the emotional pain, sadness, and humiliation. I just had another miscarriage, my third one, and I'm almost certain I'll never have children. (*Pauses for 15 seconds to think*) On top of that, my partner is growing more distant too, and I worry they might leave me for someone to have a family with.

COUNSELOR: So, you're also feeling worried about how this is going to impact your marriage and whether your partner will want to be with you.

ADEN: (*Silence for 30 seconds as she looks down and to the right*) Yes, but I realize that if my partner truly cared for me, they would not leave. I think maybe they're just as hurt and in as much pain as I am. It's my own insecurities, really. (*Pauses for 15 more seconds*) And if my partner did want to leave, then maybe they're not the right person for me to be with. Maybe I deserve better than that.

COUNSELOR: (*Silence for 20 seconds as the counselor gathers her thoughts*) So, a couple of things are at work here. First, your partner's behavior could represent grieving, and your insecurities may have distorted how you've been making sense of that grieving behavior. And second, you realize that what's important for you is to be with someone who truly and genuinely cares for you and is committed to the marriage.

Discussion Questions

1. How effective was the counselor's use of silence?
2. What alternative responses may have also been helpful?
3. Put yourself in the position of the client. What would you have been thinking to yourself during the periods of silence?

Egan and Reese (2019) developed a conceptual framework for active nonverbal listening using the acronym *SOLER*, which helps counselors remember key nonverbal techniques that convey that they are physically and psychologically present with clients:

SOLER

S: → *Squarely* face the client. Sitting facing the client gives the impression of attentiveness and engagement.

O: → Adopt an *open* posture. An open posture can show that you are available to listen, whereas crossed hands or legs can convey unavailability. Appropriate posture includes sitting with your hands either by your side or resting on your lap if you are writing.

L: → *Lean* (slightly) toward your client to convey your interest in and engagement with them.

E: → Make *eye* contact. Maintain appropriate and culturally sensitive eye contact to convey your interest and engagement.

R: → Stay *relaxed*. Try to be relaxed and natural with your client. This helps put them at ease. Fidgeting or maintaining a rigid stance can convey the impression you are not comfortable with your client.

SOLER techniques help convey a counselor's genuineness and respect toward the client and put the client at ease. When applying SOLER, reflect on the extent to which your posture communicates availability and openness to your client, and adjust accordingly. Observe yourself on video during counseling role plays. Remember to relax!

THE BASIC LISTENING SEQUENCE

Learning Objective 4.3 Identify the basic listening sequence including asking open and closed questions and reflecting skills.

The basic listening sequence represents a set of interrelated skills used to achieve three overarching goals: (1) to obtain an overall summary and understanding of the client's presenting issue; (2) to identify the key facts of the client's situation; and (3) to identify the core emotions and feelings the client is experiencing (Ivey et al., 2022). In short, these skills allow you to understand the structure of your client's story. Using these skills, you will not only convey empathy, respect, warmth, and congruence to your client but also lay the foundation for your understanding of the client's issues and the development of subsequent interventions to help achieve a successful resolution to a crisis state.

The basic listening sequence involves the ability to ask open and closed questions as well as the reflecting skills—paraphrasing, reflecting feelings, and summarizing. An explanation and overview of these skills, examples of each skill in use, and some brief exercises to help you practice these basic listening skills follow. But first, read Voices from the Field 4.1.

VOICES FROM THE FIELD 4.1

Reflective and Active Interventions

Joseph "Chip" Cooper

Crisis intervention requires us to be both reflective *and* active in our interventions, and it is important to know when to use each. Here is an example of the need to be active: I once had a client in crisis who set her shirt on fire while sitting in the waiting room as I was getting her file and preparing for the session. This was not a time to reflect and listen! We put out the fire; made sure she was not injured; and then actively assessed suicidal/homicidal intent and safety issues and developed a plan of action, which called for her to be hospitalized. I had to take an active and directive role in ensuring her safety. Crisis counseling often requires us to be quick on our feet, explore alternatives, and actively assist our clients in dealing with their problems.

Asking Open and Closed Questions

Questioning is a primary skill that allows crisis counselors to gather important and specific information about clients. Questions allow counselors to make an accurate assessment of their clients' issues and to guide and focus clients so they can make the most effective use of the counseling session. However, the use of questioning can be a double-edged sword. Used inappropriately, questioning can impede communication and block client disclosure. Drilling clients with questions can give too much control to the crisis counselor. Moreover, bombarding clients with questions could confuse and frustrate them as well as increase their level of anxiety.

Crisis counselors do not want counseling sessions to sound like an interrogation, although many of the initial intake questions are used to gather information and are

therefore necessary in crisis intervention. Counselors must be careful to appropriately pace the questions to guard against increasing clients' stress levels. Thus, crisis counselors need to be aware of how to use questions appropriately and pay close attention to the types of questions used to gather information. The two types of questions, open and closed questions, are examined next.

OPEN QUESTIONS Open questions usually elicit fuller and more meaningful responses by encouraging the client to talk at greater length. Open questions typically begin with *what*, *how*, *could*, *would*, or *why* and are useful to help begin an interview, elaborate the client's story, and bring out specific details (Ivey et al., 2022). With open questions, the client can choose the content and direction of the session and take more control. The following guidelines illustrate the use of open questions for crisis workers:

1. ***To begin the interview:*** "What would you like to talk about today?" "How can I be of help to you today?" "Tell me why you've come in today."
2. ***To elicit details:*** "Give me an example." "What do you mean by 'just give up'?" "What do you usually do when you're feeling down?"
3. ***To enrich and deepen:*** "Tell me more." "What were your feelings when that happened?" "What else is important for me to know?" "In what ways does that help?"
4. ***Focus on plans:*** "What are some things you can do to stay safe?" "Who are some people you can call when you feel this way again?" "How will you make that happen?"

Finally, be careful when using *why* questions and "leading" questions. Questions that begin with *why* often cause the client to intellectualize and can lead to a discussion of reasons. In addition, *why* questions can cause the client to become defensive, feel criticized, or feel "put on the spot." When this happens, it is not uncommon for the client to become more guarded and to shut down.

For example, think back to a time when you were younger and your parents asked the question, "Why did you do that?" How did you feel and what was your reaction? Take a moment to consider the following: "Why do you hate yourself?" versus "You say you hate yourself. Help me understand that." Which of these approaches would you prefer your counselor to use?

In addition to making clients become more defensive, *why* questions can also lead clients to feel more hopeless and despondent, as in this example:

COUNSELOR: Why do you continue to go back to a man who abuses you?

CLIENT: I know, I know. It sounds pathetic.

COUNSELOR: What draws you back to your partner? (*An alternative open question*)

Another roadblock to the use of effective questions involves leading questions. Leading questions often contain a hidden agenda because the answer or expectation is already imbedded within the question. Although well intentioned, these types of questions place too much power in the hands of the crisis counselor and tend to push the client in a preconceived direction. Here are a couple of examples of leading questions: "You did not really want to kill yourself, did you?" or "Don't you think you will feel better if you stop drinking?" Guard against the use of these types of questions. Crisis counselors want to hear a client's story as the client understands and experiences it. Open questions allow the counselor the opportunity to achieve this end without imposing values and expectations on the client. Next, complete Think About It 4.1.

THINK ABOUT IT 4.1

Think about your questioning skills. How can you use open and closed questions effectively in your work with clients during and outside of a crisis? Self-monitor conversations with friends, colleagues, students, and clients. What types of questioning techniques do you use, and in what proportion? How can you improve?

CLOSED QUESTIONS Closed questions can be used when crisis counselors need to obtain very specific, concrete information and get all the facts straight. Such questions typically either elicit a yes/no response or provide specific factual information, such as the number of drinks a client consumes in a week or the age at which the client began experiencing symptoms. In contrast to longer-term counseling, where information is gathered more slowly and the treatment plan develops over many weeks, crisis counseling often requires quick and focused responses. Thus, closed questions are very useful and in fact necessary in crisis counseling because the counselor must gather specific information to aid in the prompt assessment of the problem and development of a plan of action (James & Gilliland, 2017). Here are some examples of closed questions:

"Are you thinking of killing yourself?"

"When did these symptoms begin?"

"Do you have a family member or friend to call on when you are feeling overwhelmed?"

"How old were you when this traumatic event occurred?"

As seen in the above examples, closed questions are good for obtaining the necessary details to aid in assessment and intervention when a client is in crisis. It is not unusual for crisis sessions to integrate many closed questions for gathering pertinent information related to the crisis. It is important to obtain the details needed for an assessment before pursuing more open-ended answers since a counselor is never sure how much time one will have with a client. At the same time, crisis counselors should still guard against the overuse of closed questions, which can cause clients to shut down and simply wait for the next question to answer.

Many agencies require the completion of a crisis intake form that includes questions that a client is required to answer. One way to approach such questions and concurrently get more information from the client on the presenting issue(s) is to use a question-tree format. This format ensures that a crisis counselor asks the (surface) questions required for the crisis intake as well as follow-up questions that may provide insight into the client's issue that would contribute to a more effective crisis plan. Using this format, the counselor asks the client a required closed question first, then proceeds with specific follow-up questions that are dependent on the unique situation of the client and allow for more detail. For example:

Counselor (Initial Question):	When did you first start using cocaine?
Client:	When I was 15 years old.
Counselor (Follow-Up Question):	Tell me what was happening in your life when you were 15 years old, when you first started using.

There is a category of closed questions that is similar to leading questions and should not be used. Referred to as *negative interrogatives* (James & Gilliland, 2017), they are closed questions that are used in a subtle way to coerce the client into agreeing with you. Questions that begin with *don't*, *doesn't*, *isn't*, *shouldn't*, *aren't*, and *wouldn't* often suggest a command for the client to do something. For example, "Shouldn't you stay away from her?" or "Isn't it a good idea to stop drinking?" imply the client should agree with the crisis counselor and do something different. For counselors who want to work collaboratively with clients, a better way to ask this question would be as follows: "You stated earlier that one of the problems that brought you in today is your drinking. Would you like some information on how I can be of help to you with that?" Next, review Case Study 4.2, which shows how open and closed questions can be fluently integrated.

CASE STUDY 4.2

Effective Use of Open and Closed Questions during an Interview

The following dialogue provides a brief example of how the crisis counselor uses a blend of open and closed questions to obtain important information about Dakota, who just discovered a partner is having an affair and is planning on leaving the relationship. Dakota presented with substantial suicidal ideation.

DAKOTA: I've gotten to where I can't even sleep at night. My mind just races, and I can't stop thinking about everything.

COUNSELOR: Tell me more about some of the thoughts you have been having as you lay in bed unable to sleep. (*Open question to facilitate exploration and information gathering*)

DAKOTA: That I will never be in a happy relationship again. That my partner never really cared about me and just used me. That this pain will never stop. I wonder how I can get my life back together on my own.

COUNSELOR: You wonder if you will find peace again without your partner and want so much for this pain to go away. (*Empathic paraphrase*) What are some of the feelings you have been experiencing? (*Open question*)

DAKOTA: Mainly down, angry, and depressed. Like I said, I've been thinking of checking out—ending it all. I feel this tremendous pain inside my chest, very hurt and sad, I guess. I feel like my partner never really cared about me and is just so selfish.

COUNSELOR: You're feeling very hurt and betrayed. When did you begin experiencing these symptoms? (*Closed question to identify timeline of symptoms*)

DAKOTA: I would say about 3 months ago, when I found out about the affair, but they have gotten much worse over the last month.

COUNSELOR: You say they have gotten worse over the last month. What do you make of that? (*Open question to identify client's understanding of the progressing symptoms*)

DAKOTA: Well, when I first found out about the affair, I would talk a lot with my friends and family, but I felt like they were getting sick of hearing me complain all the time. So lately I have just been trying to tough it out and deal with it on my own.

COUNSELOR: And is this the first time you've sought counseling for this?

DAKOTA: Yes.

Discussion Questions

1. How effective was the counselor's use of open and closed questions?
2. What alternative responses by the counselor might also have been helpful?
3. Put yourself in the position of the client. What counselor responses would have elicited even better or more informative responses by the client?

As Case Study 4.2 demonstrates, the crisis counselor began with open questions to encourage exploration and to help identify the client's thoughts (e.g., "I will never be in a happy relationship again") and feelings (e.g., anger, grief, and hurt) associated with the breakup of the relationship. The crisis counselor then moved to closed questions to obtain more specific information regarding the duration of the symptoms and the counseling experience.

OPEN VERSUS CLOSED QUESTIONS As mentioned earlier, in crisis intervention work, crisis counselors often need to use closed questions to quickly identify and bring out specific details to seek the resolution of the crisis state. However, they can often obtain the same information by asking open questions, so counselors should try to refrain from moving too quickly into a closed questioning approach unless they are unable to obtain the information otherwise.

Consider the examples of closed questions and their open question counterparts in Table 4.1. Notice in the examples in the table that you can probably get all you need to know, and much more, by a subtle change in the wording of your questions to make them more open in nature. Now complete Activity 4.4 for some additional practice.

ACTIVITY 4.4

Take a moment to practice changing these questions from closed to open questions.

1. Why did you quit your job?
2. Do you think you should stop using drugs?
3. Do you get 8 hours of sleep a night?
4. Did you feel angry with them?
5. Don't you think there are other ways for you to cope with your anger?

TABLE 4.1 Open versus Closed Questions

Closed	Open
Were you afraid?	What feelings did you experience?
Are you concerned about what you will do if your partner returns?	How do you think you may react if your partner returns?
Do you see your drinking as a problem?	What concerns do you have about your drinking?
Have you thought about hurting yourself?	Tell me how you thought about hurting yourself?
Do you need help?	What help do you need?
Were you angry or sad?	How did you feel?

Reflecting Skills

Reflecting skills are a set of interventions used to help stimulate clients' exploration of their thoughts and feelings related to the presenting problems. At the most basic level, reflecting skills are a form of active listening that conveys to the client your interest in and understanding of what the client may be struggling with. Thus, reflecting skills allow counselors to convey empathy, genuineness, and acceptance to clients, which in turn facilitates the creation of a sense of safety. Moreover, reflecting skills stimulate a deeper exploration and understanding of the problem so that the client can examine the issues more objectively. The reflecting skills covered here are paraphrasing, restating, reflecting feelings, and summarizing.

PARAPHRASING When counselors paraphrase, they feed back to the client the essence of what the client just said. By paraphrasing, the counselor reflects the content and thoughts of the client's message. In other words, the crisis counselor is mirroring back to the client, in a nonjudgmental way, an accurate understanding of the client's communication and the implied meaning of that communication. Thus, paraphrasing is a reflecting skill used to convey empathic understanding and to facilitate the exploration and clarification of the client's problems (Ivey et al., 2022).

It is important for counselors to be sure the paraphrased information is accurate by checking with the client. Some clients in crisis may not feel they can refute what is being said, or their crisis state may impede their ability to completely follow the session. However, paraphrasing gives these clients permission to approve or disapprove of the accuracy of the paraphrase and its implied meaning, thereby increasing their control. In crisis counseling, adding, "Is that correct" or "Did I hear you right" allows for both accuracy and for clients to gain some control over situations where they may not have held control. When the counselor paraphrases the client's information inaccurately and does not seek affirmation by the client, the counselor may define the actual primary presenting problem inaccurately, which in turn may change the direction of the session and/or interfere with the development of the most appropriate treatment plan for the client. The importance of verifying understanding is compounded when the client needs the counselor to use a more directive approach due to the severity of the crisis.

Young (2017) proposed that reflecting skills are important because they provide the counselor with a way to do the following:

1. Communicate empathy.
2. Give feedback that enables the client to confirm or reject the impression being given.
3. Stimulate further exploration of what the client has been experiencing.
4. Capture important aspects of the client's story that may have been overlooked or covert. (pp. 123–124)

Thus, paraphrasing, if used appropriately, is a powerful therapeutic tool. Appropriate use means crisis counselors must develop the ability to take the essence of the client's statement and reflect back those thoughts and facts in *their own words*. When the counselor uses paraphrasing accurately, the client will continue to explore and elaborate. On the other hand, the counselor should not parrot back to the client what has been said. Parroting back would be a simple word-for-word restatement, not a paraphrase. Consider this example:

CLIENT: I feel so put down and disrespected by my partner. He is just like my father in a lot of ways. He was verbally abusive and full of anger. I never really felt important to him. Why do I let him treat me this way?

COUNSELOR: You feel put down and disrespected by him. Is that correct?

As you can see from the above paraphrase, the crisis worker simply parrots back what the client has said, which adds little and keeps the focus superficial. A better response would be:

COUNSELOR: Although you are trying to understand this pattern of hurt and disappointment that you've experienced from the important men in your life, it sounds like you're blaming yourself for this. Am I hearing that correctly?

To develop your paraphrasing skills, you may find it helpful to first identify the key words or content that captures the essence of your client's concern. Once you have the key content in mind, try to translate it into your own words. Examples of a client's statements, the possible key themes or words, and the resulting paraphrase follow.

EXAMPLE 1

Client: I'm so fed up with them. I try and try to get through to them, and they just shut me out.

Possible key themes or words: *fed up*, *being shut out*, *failed efforts to connect*

Paraphrase: You are at your wit's end with this. Despite your efforts to connect, you come up against a closed door. Is that correct?

EXAMPLE 2

Client: Exactly, and that's why I've been thinking about leaving the relationship. I know I deserve much better, but I just keep going back. I can't seem to make that first move.

Possible key themes or words: *leaving the relationship, being stuck, hesitation, self-worth*

Paraphrase: Although a part of you knows this is not the way you want to live your life, it is still difficult to break out of this cycle. Is that right?

As you can see from these examples, identifying the key words or themes can aid in your ability to develop accurate paraphrases that convey the essence of your client's meaning without coming across as superficial. Complete Activity 4.5 to practice your paraphrasing skills.

ACTIVITY 4.5

Think about the following client statements and try to identify the key themes or words. Then, based on these key themes or words, develop a paraphrase of your client's statement.

1. "I don't know what to do with my life. I hate my job and everything seems so meaningless. I can barely muster the energy to get out of bed in the morning. Sometimes I just want to sleep for days."
2. "I'm still in shock that my partner is having an affair. I really can't believe it. I thought we had the perfect marriage. How could I have been so stupid to not see this was happening? I feel like such a fool."
3. "I can't tell if I am coming or going. I can't sleep, I have nightmares, and I feel like a zombie throughout the day. I am so tense my body aches. No matter what I do, it just keeps getting worse."

RESTATING Often restatements can be confused with the skill of paraphrasing. In crisis counseling, the skill of restatements is intended to focus or direct the priority of the crisis and include the client's exact words. A client may be in such emotional turmoil that many themes may emerge in the session. The counselor should understand the priority is first on risk issues followed by the issues that led to the risk. The underlying issues would be secondary for crisis work and the focus of long-term counseling goals once the primary risk is identified and addressed.

For example, a crisis counselor may be working with a client who is suicidal due to an estrangement. Risk assessment (suicidal ideation) would be the primary focus in crisis work. In these situations, the counselor may need to direct the focus of the session. A restatement may be short, perhaps a single word or brief phrase to focus on the priority.

For instance, consider the following:

CLIENT: I have not been able to sleep or concentrate ever since my parent found out I was gay and kicked me out of the house. I just do not see a future for me and have been having thoughts of wanting to die.

This statement has many issues to be explored: the revelation, the estrangement, the change in baseline functioning, and the desire to die. In crisis work, the restatement would be a short phrase of the client's exact words, "wanting to die," to direct the session on the risk assessment.

Restatement: Your life has changed dramatically, and you want to die.

REFLECTING FEELINGS A wealth of research attests to the usefulness of accessing and working with feelings and emotions in counseling (Greenberg & Pascual-Leone, 2006). Naming and identifying a client's feelings can serve several important functions (Young, 2017). By reflecting feelings, a crisis counselor can help the client become aware of the emotions experienced in relation to the issue at hand. This awareness can increase the client's level of self-awareness and deepen self-disclosure. In addition, reflecting feelings can have a positive impact on the therapeutic relationship, and research has shown the quality of the therapeutic relationship to be one of the strongest predictors of counseling outcomes (Horvath et al., 2012). Moreover, it is not necessarily the specific theoretical approach of the counselor but the strength of the therapeutic relationship that is associated with the successful achievement of a client's counseling goals.

The therapeutic relationship should be characterized by an experience of mutual liking, trust, and respect between the client and the counselor. In addition, counselor qualities such as accurate empathy, unconditional positive regard, and genuineness greatly contribute to the development of the helping relationship. Thus, the reflecting skills play an important role in the development of this vital working alliance by conveying these "relationship enhancers" to a client (Young, 2017). As with paraphrasing, reflecting feelings can promote the development of accurate empathy and help to create a safe environment for the client. Also, verifying the accuracy of the reflected feelings—for example, by adding "is that correct" or "did I hear you right"—ensures accuracy and enables clients to gain some control over situations where they may not have felt in control. Now read Voices from the Field 4.2.

VOICES FROM THE FIELD 4.2

Reflecting Your Way through a Crisis

Joseph "Chip" Cooper

Clients often come to counseling in crisis. They may be feeling overwhelmed, confused, think they are going "crazy," or, in more serious cases, feeling suicidal or homicidal. In these situations, our own anxiety might lead us to become overly directive, give unwanted advice, or drill clients with questions. However, I have found doing the opposite is the key. In other words, using reflecting skills in these situations can go a long way to help a client become more emotionally regulated. Simple paraphrases, reflecting feelings, and even silence can help the client to feel understood, accepted, and safe. And clients are more likely to take the risk of changing and developing a healthy plan of action when they feel safe.

To reflect feelings, a crisis counselor must be able to recognize and put words to those feeling states observed in the client. What is the best way to practice reflecting feelings? One way is to work on becoming more aware of your own feelings and being able to accurately name them. Doing so will help you accurately recognize and name the feelings clients may be experiencing. For example, when people are asked, "How are you feeling *right now*?" the most common responses are "fine," "good," and "okay." Notice, however, that these responses do not reflect feelings and do not provide any understanding of what a person may actually be feeling.

It is important to be able to not only correctly identify and name the core feelings we all experience as humans (e.g., anger, sadness, fear, surprise, joy, love, disgust) but also accurately recognize and name our moment-to-moment feeling states that represent the finer shadings of those core emotions. For example, some of the finer shadings of the word *anger* are *irritated*, *bitter*, *enraged*, *frustrated*, and *furious*. By increasing your feelings awareness and feelings word vocabulary, you will be able to identify and respond to a client's feelings more easily and correctly. Again, due to the nature of individuals in crises, it is important to confirm the accuracy of your reflections.

How does a counselor identify a client's feelings, especially if these feelings are not explicitly stated? When clients do not state their feelings directly, counselors may still be able to infer these feelings either from the context of a client's communication or from the client's nonverbal behaviors (e.g., facial expression, posture). Thus, it is important to attend to not only what is being said but also how it is being communicated. The following practical tips will aid you in identifying and reflecting a client's feelings (Evans et al., 2017; Ivey et al., 2022):

1. To aid in identifying a client's feelings:

a. Pay attention to the affective component of the client's communication.
b. Pay attention to the client's behavior (e.g., posture, tone of voice, facial expression).
c. Use a broad range of words to correctly identify the client's emotions.
d. Silently name the client's feeling(s) to yourself.

2. To aid in reflecting feelings to a client:

a. Use an appropriate introductory phrase: "Sounds like . . . " "Looks like . . . " "You feel . . . " "It seems . . . ."
b. Add a feeling word or emotional label to the stem: "Sounds like you're angry."
c. Add a context or brief paraphrase to help anchor or broaden the reflection. This context should add the link or meaning for the perceived feeling: "Sounds like you're angry at your parents' refusal to put you in their will."
d. Pay attention to the tense. Present tense reflections can often be more powerful than past tense reflections: "You *feel* angry" versus "You *felt* angry."
e. Do not repeat the client's exact words (parroting).
f. Reflect mixed emotions: "You're feeling both angry and hurt about your parents' behavior toward you."
g. Check out the accuracy of the reflection of feeling with the client: "Am I hearing you correctly? Is that close? Have I got that right?"

Consider the following example:

CLIENT: I just sit around the house wondering what to do. We used to spend time with my partner's friends, but they know that we're no longer together. So, there is no one for me to really spend time with. I really miss them, and my own friends seem so busy. I would hate to burden them with all my problems.

COUNSELOR: You're feeling both sad and lonely right now, and you're concerned you may be just another burden on your own friends. Is that about right? (*Reflection of feeling with check for accuracy*)

CLIENT: Yes, I don't want to bring everyone down with all my problems.

There is one last point to consider when reflecting feelings. Whereas in traditional psychotherapy the focus is often on uncovering feeling after feeling by attempting to unearth a core issue, in crisis intervention work, the task is quite different. Thus, guard against going too far with uncovering feelings, as this could exacerbate the client's crisis state with overwhelming emotions. Strive for a balance of skills to build rapport, and when you do reflect feelings, be sure to keep the focus directly related to the client's presenting concerns (James & Gilliland, 2017). Complete Activity 4.6, which gives you an opportunity to practice identifying feelings so you can develop accurate reflections of feelings. Then complete Think About It 4.2.

ACTIVITY 4.6

Take a moment to read each of the following vignettes and identify the feelings embedded within the client's communications. Once you have identified the feelings, come up with your own reflection of feelings.

1. "I don't know what to do. My partner keeps working late into the night, and I feel like I never get to see them. When we do get some time together, they are moody and reserved. To make matters worse, I saw a charge on our credit card statement to a local hotel. I think they are having an affair."
2. "Ever since I was mugged, I've been having a hard time. Because of the nightmares, I can't sleep at night, and I'm exhausted all day. On top of that, I'm panicky and nervous all the time. I worry I might be losing my mind."
3. "I can't believe what my father did. He stole all the money from the trust fund my grandmother had willed to my brother and me. I've been calling him day and night, and he won't return my calls. I might have to get a lawyer, but I don't know how I'm going to afford it. Why would he do this to us?"

THINK ABOUT IT 4.2

Think about your skills in identifying and reflecting feelings. How skilled are you at identifying your feelings and the feelings of friends, colleagues, and clients? How skilled are you at reflecting those feelings? Self-monitor conversations you have over the next week and practice identifying and reflecting the feelings of others.

SUMMARIZING The final skill in the basic listening sequence is *summarizing*, a process through which a crisis counselor can begin to put together the key themes, feelings, and issues that the client has presented. By distilling the key issues and themes and reflecting this back to the client, counselors can begin to help clients make sense of what may have originally seemed to be an overwhelming and confusing experience. In addition, when clients are feeling overwhelmed and are flooded with anxiety, they will often go on tangents in many directions, making it difficult for the crisis counselor to keep up. When this occurs, brief summaries are often a useful tool to help refocus the client and reintroduce some structure into the session, which will help modulate the client's (and the counselor's) anxiety. Thus, a summary not only is used to end a session or to begin a new session by recapping the previous session, but it also can be used periodically throughout the session, helping to keep a focus and put together the pertinent issues at hand for the client.

So, when should a crisis counselor summarize? Although much will depend on the client and the content being discussed, Evans et al. (2017) offered a number of useful suggestions to help determine when a summary is in order: (1) when your client is rambling, confused, or overly lengthy in comments; (2) when your client presents a number of unrelated ideas; (3) when you need to provide direction to the interview; (4) when you are ready to move the client from one phase of the interview to the next; (5) when you want to end the interview; and (6) when a summary of the prior interview will provide you an opening to the current interview.

When summarizing, you do not have to report back to the client every single disclosed detail. This would, of course, require a prodigious memory. The key is to capture the important elements, content, feelings, and issues and to reflect these back to the client in a concise manner. The following three types of summaries are particularly relevant to crisis intervention counseling:

1. *Focusing summaries* are often used at the beginning of the session to pull together prior information the client has given and to provide a focus to the session. "Last time we met you were having trouble sleeping, having nightmares and feeling panicky throughout the day. We identified some coping skills and relaxation exercises for you to use. Tell me how these have worked for you so far."
2. *Signal summaries* are used to signal to the client that you have captured the essence of the topic and that the session can move on to the next area of concern. Signal summaries help to provide both structure and direction to the session. "So before we move on, let me make sure I understand things correctly. You discovered your partner is having an affair . . . ."
3. *Planning summaries* help provide closure and are used to recap the progress, plans, and any recommendations or agreements made. Such summaries are good for ending the session on a positive note and for providing a sense of direction for the client. "Let's take a look at what we've covered today. Ever since you were mugged, you've been having panic attacks and nightmares. We covered some coping techniques and relaxation exercises for you to practice between now and the next time we meet . . . ." (Young, 2017, pp. 161–163)

So, to put it all together, here is one more example of a summary statement: "Let me see if I understand you correctly. Yesterday you found out your son has been using cocaine for the last 6 months and has stolen money from you on several occasions. You are experiencing a mixture of feelings, especially shock and anger, and you are worried

he might turn out to be an addict like your parent was. However, you are determined to do all you can to not let that happen. How about discussing some possible directions we can go in from here?" Note that this summary captures the key issues and feelings without being too wordy and offers a transition for the counselor and client to begin identifying some action steps to take. Case Study 4.3 provides an example of reflecting skills in practice.

CASE STUDY 4.3

Integrating Paraphrasing, Reflected Feelings, and Summarizing

The following dialogue between Beck and the crisis counselor demonstrates the skills of paraphrasing, reflecting feelings, and summarizing:

BECK: I really thought things were going well with my partner, so it came as a complete shock when I found out about the affair.

COUNSELOR: You were really blindsided by this. (*Paraphrase*)

BECK: Exactly! And I've been trying to push away the pain, but I can't seem to stop thinking about it. Sometimes I just want to strangle them for putting me through this. Other times I want to die!

COUNSELOR: Even though you want so much for the pain to go away, it is still there, especially your hurt and anger. Is that right? (*Paraphrase with a reflection of feeling*)

BECK: Yes, and sometimes I can't tell which is worse, my anger or just the hurt I'm going through. I sometimes lie in bed at night and wish something terrible would happen to my ex. I'm not saying I want to kill them or anything, but I just want my ex to suffer like I'm suffering.

COUNSELOR: And this reflects the intensity of your grief right now, wanting to see your ex suffer, too. (*Reflection of feeling*)

BECK: Very true.

COUNSELOR: So, in essence, you never expected something like this to happen to you, and it has been difficult for you to tough it out and to push away the pain, grief, and anger that you have been feeling. Is that about right? (*Summary with check for accuracy*)

Discussion Questions

1. How effective was the counselor's use of paraphrasing, reflecting feelings, and summarizing?
2. What alternative responses may also have been helpful?
3. Put yourself in the position of the client. What counselor responses would have elicited even better or more informative responses by the client?

DEESCALATING CLIENT EMOTIONS

Learning Objective 4.4 Describe basic deescalation skills to help clients regulate emotions.

There are four primary objectives when counseling agitated clients: safety, client emotion management to regain control, avoidance of physical restraint, and avoidance of escalating coercive interventions. Richmond et al. (2012) reviewed the literature on verbal deescalation of agitated patients and suggested the following guidelines for environment, people, and preparedness:

- Physical space should be designed for safety.
- Staff should be appropriate for the job.
- Staff must be adequately trained.
- An adequate number of trained staff must be available.
- Objective scales should be used to assess agitation (pp. 18–20)

Richmond et al. (2012) also determined that 10 domains of deescalation exist that help practitioners in their care of agitated clients. These domains and the key recommendations associated with them appear in Table 4.2.

TABLE 4.2 The Domains of Deescalation and Associated Recommendations

Domain	Recommendations
Domain 1: Respect personal space.	Respect the client's and your personal space.
Domain 2: Do not be provocative.	Avoid iatrogenic (body language) escalation.
Domain 3: Establish verbal contact.	Have only one person verbally interact with the client. Introduce yourself to the client and provide orientation and reassurance.
Domain 4: Be concise.	Be concise and keep it simple. Repetition is essential to deescalation.
Domain 5: Identify wants and feelings.	Use free information to identify wants and feelings.
Domain 6: Listen closely to what the client is saying.	Use active listening.
Domain 7: Agree or agree to disagree.	
Domain 8: Lay down the law and set clear limits.	Establish basic working conditions. Limit setting must be reasonable and done in a respectful manner. Coach the client in how to stay in control.
Domain 9: Offer choices and optimism.	Offer choices. Broach the subject of medications. Be optimistic and provide hope.
Domain 10: Debrief the client and staff.	Debrief the client. Debrief the staff.

Source: Based on Richmond et al. (2012).

Types of Aggression

It is important to remember that the association between agitation and aggression is not fully understood in causal terms (Nordstrom & Allen, 2007). Richmond et al. (2012) summarized four different types of aggression and suggested that each type required a different style of intervention: (1) instrumental aggression, (2) fear-driven aggression, (3) irritable aggression based on boundary violations, and (4) irritable aggression based on chronic anger. *Instrumental aggression* is generally used by people who have learned that they can get their way through threats or actual violence. So, if a client makes an overt threat, unspecified counteroffers should be used. For example:

CLIENT: If I don't get what I want, I'm gonna hurt someone bad.

(*Might be countered with . . .*)

COUNSELOR: That is not a good idea.

CLIENT: Why not?

COUNSELOR: Let's not find out. Now, you were saying that . . .

In *fear-driven aggression*, the client is on the defensive and may behave aggressively to protect against being hurt. In such situations, the counselor should give the client plenty of personal space, three to four times the normal space, and match the client's pace and rhythm until the client is fully focusing on the content of the discussion rather than the internal state of fear. For example:

CLIENT: I'm so angry! I'm so angry!!

COUNSELOR: I can help you calm down and fix things. I can help you calm down and fix things.

Irritable aggression stems from two causes: boundary violations and chronic anger. *Irritable aggression based on boundary violations* is usually displayed by individuals who have experienced some humiliation or interpersonal violation and are trying to put their world back into some semblance of order. For example, a close friend may have betrayed the client and the client is struggling to regain self-integrity and self-esteem while perceiving that life and other individuals are unfair. Usually, the best approach in this instance is to agree with the client that anger is a justifiable emotion, in principle, but that the resolution to the problem cannot occur while enraged. For example, "I want to know more about your situation and help you, but I can't until you get control over your emotions so we can talk about it and figure out what to do and what not to do."

Finally, *irritable aggression based on chronic anger* is displayed by clients who are angry at the world and seem to be looking for ready excuses to "lose it." Their behavior is characterized by erratic and unrealistic demands aimed at relieving the pressure they allow to build up. They create fear and confusion through feigned or real attacks. When interacting with an escalated, chronically aggressive client, it is essential not to appear defensive or startled but to respond in an emotionless manner, repetitively offering choices and options for resolving or deescalating the situation. Let the client know that when they are willing to cooperate, you are more than willing to work with the client. Set firm limits but be prepared to engage in physical restraint, if required. Unfortunately, chronically aggressive patients often push the limits by doing the opposite of what you have asked.

Handling Aggression

The National Association of Social Workers (NASW; 2017) notes that reasoning with angry, out-of-control people is not likely possible. In such cases, deescalation becomes the primary goal. Reasoning and problem solving are possible only after deescalation occurs. Deescalation requires the counselor to remain calm, even when one is frightened and angry. Professional counselors are well advised to practice deescalation interventions repeatedly until they become automatic. NASW provides recommendations on a practitioner's physical stance and verbal interventions when faced with an aggressive client.

Regarding physical stance:

1. Never turn your back . . .
2. Always be at the same eye level . . .
3. Allow extra physical space . . .
4. Do not stand full front to client. Stand at an angle so you can sidestep away if needed.
5. Do not maintain constant eye contact.
6. Do not point or shake your finger.
7. DO NOT smile. This could look like mockery or anxiety.
8. Do not touch . . . physical contact [is easily misinterpreted] as hostile or threatening.
9. Keep hands out of your pockets, up and available to protect yourself . . .
10. Don't be defensive, judgmental . . . or parental . . . . (pp. 1–2)

Regarding verbal deescalation interventions:

1. . . . [C]almly bring the level of arousal down . . .
2. Do not get loud or . . . yell . . . Wait until [they] take a breath; then talk. Speak calmly at an average volume.
3. Respond selectively; answer all informational questions [but] . . . DO NOT answer abusive questions.
4. Explain limits and rules in an authoritative, firm, but always respectful tone. Give choices . . .
5. Empathize with feelings but not with the behavior . . .
6. Suggest alternative behaviors where appropriate . . .
7. Give the consequences of inappropriate behavior without threats or anger.
8. Represent external controls as institutional rather than personal.
9. If you assess that de-escalation is not working, STOP! You will know within 2 or 3 minutes . . . Tell the person to leave, escort [them] to the door, call for help, or leave . . . and call the police. (p. 2)

Defusing Through Reflection

Finally, at a more basic level, when reflecting feelings, a useful way to help defuse a client in an emotional crisis state is to also reflect the client's stated, or unstated, need or goal that is implied within the context of their narrative. Because of the interpretive aspects, checking for accuracy is also important with this skill. The formula for this would be: "You feel _____ because _____ and you want (or need) _____." For example:

CLIENT: Every time I do something for her, it's not good enough and she puts me down. (*Yelling*) It's all I can do to keep from killing her!

COUNSELOR: You're *feeling* so angry and hurt right now *because* she puts you down, and you really *want* to be more respected and valued in your relationship. Is that correct?

Here is another example:

CLIENT: (*Sobbing, holding herself, and rocking and shaking*) I can't take it anymore. After my husband and daughter were killed in the car accident, everything just fell apart for me. I just think maybe I should be dead too.

COUNSELOR: You're *feeling* so much pain and grief *because* of the loss of your family, and you want so much to find your own *peace* now. Is that how you feel?

Deescalation is about transferring calmness to the client and showing genuine interest in what the client has to say. Be respectful, set clear limits, and trust that you have supplied the conditions that allow the client to want to respond positively. If the client possesses a weapon, do not be a hero and try to deescalate the situation; simply comply (NASW, 2017). In a crisis, the above interventions can go a long way to helping your client feel safe with you, to diffuse overwhelming emotions, and to set the groundwork to begin working on a plan of action.

SUMMARY

This chapter reviewed the basic counseling skills used in crisis intervention work. Using these skills will help counselors develop a relationship with clients and create a safe therapeutic environment. Nonverbal attending behaviors such as appropriate eye contact, body position, and tone of voice communicate interest, warmth, and understanding to the client. Be sure to face the client and adopt an open, relaxed, and attentive body posture while maintaining culturally appropriate eye contact. Tone of voice should be steady and clear and should be used to convey a sense of safety, warmth, and security for the client.

The skills covered in the basic listening sequence include asking open and closed questions, paraphrasing, restating, reflecting feelings, and summarizing. The basic listening sequence enables counselors to gather important information about the client's issues and to develop trust and rapport. This allows the crisis counselor to begin the collaborative process of determining a plan of action for the client. Open and closed questions are used to gather information, to aid in assessment, and to provide focus and direction to the session. Paraphrasing is used to mirror back to the client, in a nonjudgmental way and in one's own words, an accurate understanding of the client's communication. A restatement is intended to focus or direct the priority of the crisis and include the client's exact words. Reflecting feelings is used to convey an understanding of the client's emotional experience. In crisis situations, the use of these skills should be checked for accuracy. Summarizing puts together the pieces, helping the client make sense of what may have originally seemed to be an overwhelming and confusing experience. Summarizing can also be used to keep the session focused, to provide direction to the interview, and to provide closure to the session by reviewing the progress, plans, and any recommendations/agreements made. By using these skills appropriately, you create the necessary conditions for positive change.

Four primary objectives when counseling agitated clients include safety, client emotion management to regain control, avoidance of physical restraint, and avoidance of escalating coercive interventions.